

Interventional Physical Medicine & Rehabilitation, P.C.

Vadim Abramov, M.D. | Physiatry

451 Main Street, 2nd Floor, New Rochelle, NY 10801

Initial physiatry evaluation

Patient: Justin W. Sapini

DOB: 12/21/1995

Accident date: 04/23/2023

Visit date: 05/04/2023

Male | 5 ft 11 in | 195 lbs | Right-hand dominant | Age 27 at accident

Address at accident: 7 Valley Drive, Nanuet, NY 10954

Financial advisor, Northwestern Mutual, commission only; out of work since accident

Coverage history: Progressive no-fault via own auto policy; later Medicaid

HISTORY

Motor vehicle accident on 04/23/2023, driver, sideswiped on the driver's side.

PRESENTING SYMPTOMS

Neck and upper back pain with left shoulder complaints. Lower back symptoms reviewed. Patient reports difficulty with sustained sitting and reaching.

EXAMINATION

Alert and cooperative. Cervical and lumbar movement limited by discomfort. Left shoulder active elevation painful. Gross distal motor function intact.

ASSESSMENT AND PLAN

Persistent spinal and left shoulder complaints following the reported accident. Continue coordinated conservative care and orthopaedic evaluation. Review forthcoming imaging when available.

Clinician / record author: Vadim Abramov, M.D.

Signature: _____ Date: _____

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451 Main Street, 2nd Floor, New Rochelle, NY 10801

Post-operative physiatry follow-up

Patient: Justin W. Sapini

DOB: 12/21/1995

Accident date: 04/23/2023

Visit date: 09/07/2023

Male | 5 ft 11 in | 195 lbs | Right-hand dominant | Age 27 at accident

Address at accident: 7 Valley Drive, Nanuet, NY 10954

Financial advisor, Northwestern Mutual, commission only; out of work since accident

Coverage history: Progressive no-fault via own auto policy; later Medicaid

HISTORY

Motor vehicle accident on 04/23/2023, driver, sideswiped on the driver's side.

INTERVAL HISTORY

Left shoulder arthroscopy performed by Dr. Capiola on 07/26/2023. Residual shoulder discomfort with ongoing neck and lower back complaints.

AVAILABLE FINDINGS

Cervical MRI: straightening of lordosis and shallow C5-6 bulge flattening the left ventral canal margin. Lumbar MRI: L5-S1 bulge with lateral predominance and bilateral lateral recess/inferior foraminal narrowing.

ASSESSMENT AND PLAN

Continue rehabilitation within postoperative precautions and coordinated follow-up with orthopaedics. Review activity tolerance and the established spinal care program. No additional procedure performed at this visit.

Clinician / record author: Vadim Abramov, M.D.

Signature: _____ Date: _____

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